Clinical Guideline for:
Pregnancy Loss
Care of women who experience Mid-Trimester Loss from 14/40 – 23+6/40

Eastern and Northern Maternity Services

Summary

This guideline contains information for Midwives, Nurses and Medical staff who are required to care for women experiencing mid-trimester loss. Guidance covers clinical care as well as support services/psychological care. This guideline is also relevant to departments Trust-wide, e.g. the Emergency Department.

Key Points

The essential elements of this guideline are:

- The importance of caring for the woman and her family in an empathetic and sensitive manner from diagnosis to discharge.
- Clear guidance in the care of the woman during labour, birth and postnatal care.
- Clear guidance for completing the documentation following delivery.
- Information on investigations.
- The role of the mortuary and options for funeral services.
- Wherever practicably possible, healthcare professionals should aim to put the needs of the mother and partner as the highest level of priority.
- Healthcare professionals should use the terminology preferred by those experiencing the loss when communicating with them.
- Due to shock and distress, information given may not be absorbed and may need to be repeated or provided in writing (or other formats).
- Differences between people's culture, ethnicity, age and socioeconomic status are likely to influence communication. Some may find it less easy to ask questions or express their wishes.
- Use warm, open body language by sitting near to the parents, facing them, making eye contact and using touch if appropriate. Be mindful of your tone and any background noise. Be honest whilst being as sensitive as possible. It is ok to show emotion.

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1. INTRODUCTION

1.1 This clinical guideline is designed to provide advice, guidance and direction to staff whilst leaving room for professional judgement, and adaptation, to fit individual circumstances.

1.2 This guidance covers mid-trimester loss, i.e. intra uterine fetal death (IUFD), miscarriage and medical termination of pregnancy for fetal abnormality (TOPFA) between 14+0 weeks and 23+6 weeks gestation.

1.3 Women who present to the Emergency Department

- From 14 weeks gestation on the Eastern site.
- From 18 weeks gestation for the Northern site.

Should be referred immediately to Labour Ward.

1.4 In addition to any physical effects, mid trimester loss often has a profound emotional, psychiatric and social effect on the parents, their relatives and friends.

1.5 SANDS (Stillbirth and Neonatal Death Charity Society) are a charity that provides support for grieving families and educational programmes for professionals. SANDS have produced the 9 Principles of Bereavement Care. They have also supported the development and introduction by the NBCP of a set of standards that underpin the care given to ensure that care is sensitive and standardised throughout the country. Should a pregnancy loss occur this guideline aims to support staff in caring for these women at a very difficult time.

2. BACKGROUND

2.1 The purpose of this document is to detail the process for the care of women and their families experiencing pregnancy loss between 14 weeks gestation and 23+6 weeks gestation, following the principles set out by SANDS, (the Stillbirth and Neonatal Death Charity) together with their recommendations for improving bereavement care. It is also to be used in conjunction with the standards recommended by the National Bereavement Care Pathway (NBCP) for Miscarriage, Stillbirth, Neonatal Death and Termination of Pregnancy for Fetal Anomaly (TOPFA).

2.2 The policy applies to all Trust staff that are likely to come into contact with bereaved parents, most particularly staff working in the Maternity Unit. Staff working within other departments, e.g. A&E and KGV / Wynard must also be aware and adhere to this guideline when caring for bereaved parents.

2.3 Implementation of this policy will ensure that:

- Parents receive sensitive individualised care during the loss of their baby.
- Parents receive a parent-led bereavement care plan which provides continuity between settings.
- The correct procedures are followed and the appropriate forms are completed by care givers within the Trust.

3. DEFINITIONS

3.1 Viability

3.1.1 In the UK, the legal age of viability is set at 24+0 weeks gestation. This is the gestation at which a fetus is considered potentially able to survive outside of its mother.

3.1.2 Any questions about gestation on admission should be managed by a thorough and clearly documented Obstetric and Neonatal assessment.

3.1.3 Any decisions about resuscitation at the cusp of viability should be directed to the Consultant Neonatologist on-call taking in to consideration the exact gestation, estimated fetal weight (if known) and the wishes of the parents.

3.2 Late Fetal Loss

3.2.1 A baby delivered with no signs of life, from 18 weeks to 23+6 weeks gestation, irrespective of when the death occurred.

3.3 Stillbirth

3.3.1 The RCOG states that the legal definition of stillbirth is:

"a baby delivered with no signs of life known to have died after 24 completed weeks of pregnancy".

This will include:

- A baby delivered with no signs of life, at or after 24+0 weeks gestation, irrespective of when the death occurred.
- Antepartum Stillbirth: A baby delivered with no signs of life, known to have died in utero before the onset of labour after 24+0 weeks gestation.
- Intrapartum Stillbirth: A baby delivered with no signs of life, known to have been alive at the onset of labour after 24+0 weeks gestation.

3.4 Neonatal Death

3.4.1 The death of a live born infant, regardless of gestational age at birth, within the first 28 completed days of life. Even babies born before 23 weeks gestation can show signs of life and would then be a neonatal death (please see signs of life criteria 15.1).

This does not include babies born before 23 weeks gestation that show physiological reflex activity within the first minute following birth. These include:

- Short gasps.
- Brief visible pulsation of the chest wall.
- Twitches or involuntary muscle movement.
- Any of these reflex movements within the first minute following birth, that then stop, do not constitute signs of life.

4. DIAGNOSIS – please see flow chart (Appendix 1)

4.1 Real time ultrasound – allows direct visualisation of the fetal heart.

Absence of fetal cardiac activity should be confirmed by 2 qualified clinicians (this can include sonographers, Consultant Obstetricians, Middle-Grade Doctors and Midwives or Nurses with a professional ultrasound qualification).

4.2 Staff should be aware that parents are often highly sensitive to non-verbal messages and body language during scans. They may become alarmed if the screen is turned away or if a colleague is called into the room. If a reason for concern is found, the sonographer should tell the woman as soon as possible that there is something on the scan that needs to be looked at more closely. It is important not to pretend that all is well.

4.3 Auscultation **should not** be used to confirm suspected IUFD.

4.4 Parents are often not expecting to hear difficult news during a scan and so should be given time to absorb the results before staff offer to discuss the next steps.

5. DISCUSSING DIAGNOSIS AND SUBSEQUENT CARE

5.1 If the woman is unaccompanied, an immediate offer should be made to call her partner, relatives or friends and moved to a quiet room in an appropriate location, to discuss ongoing management of care.

5.2 Following diagnosis the woman and her family may be shocked and unable to process the information. Some may feel guilty or ashamed that they had not realised that anything was wrong.

5.3 Breaking what is likely to be bad news is extremely difficult for staff. Parents should be told as soon as it is suspected that something is wrong, even if it is not yet confirmed. Parents often sense something is wrong due to the reactions of staff which will only cause additional distress and anxiety.

5.4 Use clear, straightforward communication. Do not be ambiguous, or use euphemisms.

5.5 The preferences of all bereaved families should be sought and all bereaved parents should be offered informed choices about decisions relating to their care and the care of their babies.

Mid-trimester loss poses particular difficulties as it is often sudden and unexpected. A crucial component is to determine the emotional feelings of the mother and her companions. This empathetic approach seeks to identify and understand women's thoughts and wishes but without trying to shape them. Pregnancy loss can quickly result in vulnerability and imposing care can worsen the psychological impact. Decisions about care will vary widely from individual to individual. Carers should neither persuade parents or make assumptions that would limit parental choice, but just listen, make eye contact and sit with them if needed.

5.6 If the woman had been aware of fetal movements before the baby's death, she should be told that she might still feel sensations similar to fetal movement even though the baby has died.

5.7 For families where communication is difficult due to language barriers, an interpreter should be used. Using family and friends is strongly discouraged. Parents may not want the informal interpreter to know everything about them. A family member or friend may also be deeply affected and their ability to interpret may be affected. Never use a child or young person under the age of 16 years unless immediate emergency care is required and no other options are available.

5.8 Multiple pregnancies

When one or more babies from a multiple pregnancy has died, parents may be advised to continue the pregnancy for the benefit of any surviving babies. Waiting to give birth may be difficult for parents who feel fearful for their surviving baby. Some parents may also be distressed about the idea of the dead baby remaining with any surviving baby or babies. However, other parents may find this thought comforting. Additionally, some parents may feel that they are unable to start grieving for their baby or babies who have died until after birth. Some parents may also have concerns that their fear and grief may affect the surviving baby or babies. Staff should acknowledge the potential difficulties for parents who have a multiple pregnancy where one or more babies have died and offer parents emotional support. Support from staff will be particularly important if some family members and friends discount the reality of the loss and focus only on the surviving baby or babies.

5.9 The Midwife/Nurse will need to inform the parents of specific aspects of care following the birth, i.e.:

- Being able to see / spend time with their baby. This time together should not be limited or restricted.
- Mementos, i.e. photographs, hand and footprints (see Appendix 3).
- Religious and cultural care, including support at the time of birth and the possibility of blessing and/or naming the baby and advice about funerals.

5.10 All parents should be asked if they would like pastoral, religious or spiritual support from the Chaplaincy team.

Inform parents of the bereavement room and offer to visit room with them if it's available.

6. INVESTIGATIONS OF CAUSE OF MIDTRIMESTER LOSS

6.1 Clinical assessment and laboratory tests should be recommended to assess maternal wellbeing and to determine the cause of death, the chance of recurrence and possible means of avoiding future pregnancy complications.

6.2 Parents should be advised that no specific cause is found in almost half of mid-trimester losses.

6.3 Parents should be advised that when a cause is found it can crucially influence care in a future pregnancy.

6.4 Carers should be aware that an abnormal test result is not necessarily related to the mid- trimester loss; correlation between blood tests and post mortem examination should be sought. Further tests might be indicated following the results of the post-mortem examination.

7. TESTS RECOMMENDED TO IDENTIFY THE CAUSE OF MIDTRIMESTER LOSS

7.1 Tests should be directed to identify scientifically proven causes of mid-trimester loss. This can be done by completing the pregnancy loss order set on EPIC.

7.2 Blood tests are not required if having medical termination for fetal abnormality.

8. WOMEN WHO ARE RHESUS D NEGATIVE

8.1 If equal to or above 20 weeks gestation, **all** women should be advised to have a Kleihauer test undertaken urgently to detect large fetal-maternal haemorrhage (FMH) that might have occurred a few days earlier.

8.2 Anti-Rh D immunoglobulin should be administered to all rhesus negative women as soon as possible after presentation. If free fetal DNA has shown the fetus to be Rhesus negative then Anti D is not required. Maternal blood taken in the first trimester will confirm if the baby is Rh D.

8.3 If there has been a large FMH, the dose of anti-Rh D immunoglobulin may need to be larger than usual. Blood transfusion will advise on the dosage and whether a repeat Kleihauer is required.

9. LABOUR AND BIRTH – please also see flowchart (Appendix 2)

9.1 Recommendations about labour and birth (or medical termination) should consider the mother's preferences as well as her medical condition and previous obstetric/intrapartum history.

9.2 Women should be strongly advised to take immediate steps towards delivery if there is sepsis, pre-eclampsia, placental abruption or membrane rupture, but a more flexible approach can be discussed if these factors are not present. In the event of sepsis, commence Sepsis Care Bundle.

9.3 Well women with intact membranes and no evidence of DIC should be advised that they are unlikely to come to physical harm if they delay labour / medical termination for a short period, but they may develop severe medical complications and suffer greater anxiety with prolonged intervals. Women who delay labour for periods longer than 48 hours should be advised to have testing for DIC twice weekly.

9.4 Women contemplating prolonged expectant management, parents should be advised that the value of post-mortem may be reduced and that the appearance of the baby may deteriorate.

9.5 If a woman returns home before labour, she should be given a 24-hour contact number for information and support - Triage 01392 406616.

9.6 Syntometrine / syntocinon is recommended for all women from 14 weeks gestation for the third stage of labour, however, the Consultant may decide to continue with Misoprostol.

10. INDUCTION OF LABOUR (IOL) / TERMINATION OF PREGNANCY (with unscarred uterus)

- Mifepristone 200mg Orally, 48 hours prior to admission.
- Followed 48 hours later by Misoprostol 400mcg PV/PO/sublingual/Buccal every 4 hours until delivery.

10.1 If a woman is undergoing medical or expectant management and has decided to go home, she should be offered verbal and written information about what to expect, managing pain and bleeding, whom to contact and what to do in an emergency.

11. INDUCTION OF LABOUR / TERMINATION OF PREGNANCY (with history of LSCS)

- A discussion of the safety and benefits of IOL should be undertaken by a Consultant Obstetrician.
- Mifepristone can be used alone to increase the chance of labour significantly within 72 hours (avoiding the use of prostaglandin).
- Women with a single lower segment scar should be advised that, in general, IOL with prostaglandin is safe but not without risk.
- Women with 2 previous CS deliveries should be advised that in general the absolute risk of IOL with prostaglandin is only a little higher than for women with a single previous CS.
- Women with more than 2 CS deliveries or atypical scars should be advised that the safety of IOL is unknown. In these situations, an individual plan of care should be made by the Consultant.
- Therefore:
  - Mifepristone 200mg once daily for 2 days. Admission 48 hours after 2nd dose.
  - Misoprostol 400 mcg PV/Sublingual/Buccal every 4 hours for four doses. Senior review is then required.

Senior review is required after 4 doses.

In women with >1 LSCS an individual plan should be made by the Consultant Obstetrician.

12. WOMEN LABOURING WITH A SCARRED UTERUS

12.1 Women undergoing VBAC should be closely monitored for features of scar rupture.

12.2 Oxytocin augmentation can be used for VBAC, but the decision should be made by a Consultant Obstetrician.

13. PLACE OF BIRTH

13.1 Women should be advised to labour in the Hospital.

13.2 The Bereavement Room is available on labour ward for women experiencing pregnancy loss. In the event that the bereavement suite is not available, a suitable alternative room should be sought on labour ward.

Labour Ward
East – below 14 weeks will be cared for on Wynard Ward
North – Below 18 weeks will be cared for on Petter or KGV

14. PAIN RELIEF

14.1 Discuss individual requirements for pain relief. If necessary discus with the Anaesthetist.

15. RESPONSIBILITIES IF BABY SHOWS SIGNS OF LIFE AT BIRTH

15.1 MBRRACE definition of 'signs of life' include one or more of the following:

- Easily visible heartbeat seen through the chest wall.
- Visible pulsation of the cord after it has been clamped.
- Breathing, crying or sustained gasps.
- Definite movement of the arms and legs.

If signs of life are thought to be present the neonatal team should be called. This will only include babies born at the cusp of viability between 23+1 and 23+6 weeks that have been assessed by the neonatal team on call and where a decision for active resuscitation has been made. A medical certificate of cause of death (death certificate) can only be issued if a Doctor has seen the baby alive. It is best in these cases for the Obstetrician to be called as calling the Neonatal Consultant may cause confusion for the family. If a Doctor has not seen the baby alive, the death will require a referral to the Coroner, this can be discussed with the Medical Examiner.

15.2 A baby would not be considered for resuscitation before 22 weeks gestation. From 22 weeks to 23+6 weeks a discussion would be necessary between Consultant Obstetrician, Consultant Neonatologist and parents.

15.3 The baby should be treated with dignity and respect. The parents may be distressed and will require support and guidance from the Midwife according to their wishes.

15.4 Babies born with signs of life should be seen by a Doctor at the earliest opportunity, so that in the event of subsequent death a neonatal death certificate may be issued to the mother after discussion with the medical examiner.

15.5 Following a neonatal death please contact the Medical Examiner Office to notify them. They cover both sites and will be able to guide staff through the certification process. The medical certificate should not be completed until discussion with the Medical Examiner Office. They can be contacted between the hours of 8am - 4pm Monday to Friday, outside these hours please leave a voice message.

RDUH medical examiner contact details: 01392 408396

15.6 Neonatal death at any gestation should be notified to the Child Death Coordinator and an MBRRACE notification completed to trigger the cascade to CDOP. There is a list of those authorised people to complete the MBRRACE notification in the help folders.

Child Death Coordinator: Kristy Forristal. Kristy.forristal@nhs.net

16. FOLLOWING THE BIRTH (see Appendix 3)

16.1 Staff must maintain the dignity of the baby who has died.

16.2 Following the birth, parents should be asked whether they would like to spend time with their baby. This can either be alone or with family members / siblings / friends. The time spent will vary with some parents only wanting to look at their baby whilst others may want to keep the baby with them for a period of time.

16.3 When parents wish to see their baby, the baby should be dressed or wrapped either in clothes / blankets brought in by the parents or provided by the department and chosen by the parents. Cuddle cots are recommended for use in a crib or whilst baby is being held to ensure baby remains cool.

16.4 Ensure parents are offered mementos and given the opportunity to participate in making keepsakes/memories of their choice.

16.5 Parents may wish to take their baby home or directly to a funeral director. In these circumstances, please complete the 'Taking the Deceased Baby Home by Parents' form on EPIC, and print a copy for the parents.

17. BABY DETAILS

17.1 The baby should have an identification label, (If too small attach to the cord clamp). A copy of the labels can be offered to the parents as a memento.

The baby must be referred to, and identified by, the mother's surname whilst in hospital.

If it is requested that the baby undergoes post mortem, the baby will need to be sent to another Trust that offers a Perinatal Pathology Service. In this circumstance, the baby's ID details will be the same as the mothers.

17.2 Identification of sex of baby

a) For babies of twenty weeks gestation and over the sex should be identified clinically.

b) For babies of under twenty weeks' gestation the sex can only be confirmed by pathological examination.

East contact: FMAU (Ext. 6540) for further advice.
North Contact: EPAC (Ext. 2722) for further advice.

18. GUIDANCE FOR CYTOGENETIC ANALYSIS OF THE BABY

18.1 Verbal consent should be taken and recorded for any placental samples used for karyotyping.

18.2 Women undergoing a TOP for a proven chromosome abnormality do not need chromosomes resent or a post mortem.

18.3 If the baby is having a post mortem examination the placenta should be placed in a specimen pot with formalin and kept with the baby. If there is no post mortem the placenta should be placed into formalin and sent to the histology department. If cytogenetics is indicated, samples must be taken before formalin is added and sent with 2 x completed forms.

Babies who are having a full post-mortem do not need a separate cytogenetic sample as this will be taken via a skin sample during the PM.

If you are in any doubt, please contact the mortuary for advice. If out of hours and you are in doubt, send the placental fresh without formalin and notify the mortuary. They will add the formalin if required.

18.4 Please do not routinely offer karyotype to all pregnancy loss women as many are rejected from the lab quoting the eligibility criteria laid out in the National Genomics Test Directory (NGTD).

They will only process if there are clear abnormalities and we highlight that on the form or it is a third trimester loss.

Please ensure you are aware of these guidelines so we are not having to apologise to women in our debriefs.

18.5 Fetus with a likely chromosomal abnormality

Testing criteria. This indication is relevant to:

- Ongoing pregnancies
- Where there has been fetal loss, termination of pregnancy or miscarriage, accompanied by additional features suggestive of chromosomal abnormality OR
- Third trimester intrauterine death or stillbirth in the absence of other likely causes.

19. PERINATAL POSTMORTEM EXAMINATION

19.1 Consent for post mortem examination should be sought by a member of staff who has received training in seeking post mortem consent and the process and who has a good understanding of the procedures for which they are seeking consent. The discussions should not be hurried (a minimum of one hour should be allocated for this process). Parents should be offered full post-mortem examination to help explain the cause of mid-trimester loss but also that no definite cause of death may be found. A post mortem may not be required if it is a termination due to known fetal abnormality – Please check with the Consultant.

19.2 Parents should be advised that post-mortem examination provides more information than other (less invasive) tests and can sometimes be crucial to the management of future pregnancy.

19.3 Attempts to persuade parents to choose post-mortem must be avoided; individual, cultural and religious beliefs must be respected.

19.4 Written consent must be obtained for any invasive procedure on the baby. Consent should be sought or directly supervised by an Obstetrician or Midwife trained in special consent issues and the nature of perinatal post-mortem, including retention of any tissue for clinical investigations, research and teaching explained. A post mortem consent form must be completed, photocopied and provided to the parents, sent to be scanned on to EPIC and the original sent to the mortuary with the baby.

19.5 Parents should be informed that post mortems are carried out at a Centre of excellence, i.e. Bristol and that their baby is usually gone for a week but may be gone for up to three weeks. They will be informed by the mortuary staff when their baby has been returned to the hospital mortuary. The results may take several months to be received.

19.6 A **Perinatal post mortem request form (Bristol)** should also be completed, photocopied and sent with the baby. Copies of USS should be attached to the form accompanying the baby. Two registered staff should check that the details entered on the form are correct. The request form needs to be sent for scanning onto EPIC once the family are discharged.

19.7 Parents should be offered a description of what happens during the procedure and the likely appearance of the baby afterwards. This should include information on how the baby is treated with dignity and any arrangements for transport. Parents will be given a time period to allow them the opportunity to change their mind about consent for post mortem. Histological examination of the cord, membranes and placenta should be recommended whether or not post-mortem examination of the baby is requested.

19.8 Parents who decline full post-mortem might be offered a limited examination, but this should be discussed with the perinatal pathologists before being offered. Placental examinations should also be discussed with all parents and, wherever possible, all placentas from babies who have died should be sent for examination by a Specialist Pathologist.

20. DOCUMENTATION

20.1 There are folders available for pregnancy loss <24 weeks. The folder contains all the necessary paperwork and documentation that is required and the help folder gives staff guidance on how to complete all the relevant paperwork and what is required to be completed on EPIC. If the Bereavement Midwife or Labour Ward Coordinator is available they will provide support for staff completing the documentation. All clinical care should be documented on EPIC. A scanned copy of the completed consent form should be uploaded to the mothers record on EPIC.

21. PMRT

21.1 A Perinatal Mortality Review Tool (PMRT) will be completed for:

- All late fetal losses 22+0 to 23+6
- All neonatal deaths from 22+0 to 28 days after birth.

These cases should be reported on Datix to alert the Governance team to notify the case through the electronic PMRT portal. A PMRT Data Collection form must be completed for these cases. The Bereavement Midwife will collect these regularly. An initial investigation will be commenced which will inform a multidisciplinary panel review. The review panel will generate a report and an action plan that can be shared with the parents during a Consultant de-brief appointment. Please ensure parents are provided with the PMRT information sheet and invited to input into the process. The Bereavement Midwife will contact them to support and submit any questions they have.

21.2 The PMRT process also includes the option for parents to feedback. Good feedback mechanisms provide parents with opportunities to inform service improvements and to feel listened to. It can also be beneficial for the staff who cared for the family to know that the care they provided was beneficial.

22. TRANSFER OF THE BABY TO THE MORTUARY

22.1 Before storage or transferring a baby, the baby should be labelled with the mother's name and NHS number or unique identifier, the baby's date and time of death, the baby's name if they have one, the baby's sex (if known) and the name and details of the person who verified the death. Items accompanying the baby should also be listed.

22.2 Babies transferred to the mortuary must be wrapped in an incontinence sheet cut to size and a small cotton sheet to prevent warming and then into a small transport casket. Any clothes/toys/blankets should go in the body bag with casket.

22.3 All green documentation should be placed in an envelope (with the mother and baby details on the front) and transferred to the mortuary with the baby. The envelope should be placed on the clip board next to the entry register.

22.4 Complete a **Baby Notice of Death** on EPIC with details required for the mortuary register.

22.5 The baby must be transferred to the mortuary by a Porter, accompanied by a Midwife if the family wish. If parents request to escort their baby to the mortuary, this can be arranged with prior consultation of the mortuary staff.

23. POSTNATAL

23.1 Women should be cared for in the bereavement room on labour ward (or a single room if being cared for on the gynaecology ward) until they are discharged.

23.2 If there is already a woman in the bereavement room, consider transferring her to the gynaecology ward if there is a need for her to remain in hospital, or an alternative suitable Labour Room allocated.

24. THROMBOPROPHYLAXIS

24.1 Stillbirth is a postnatal risk factor for VYE and women should be routinely assessed for thromboprophylaxis.

24.2 Thromboprophylaxis should be discussed with a haematologist if the woman has DIC.

25. SUPPRESSION OF LACTATION

25.1 Women should be advised that almost one third of those who choose non-pharmacological measures are troubled by excessive breast discomfort.

25.2 Cabergoline 1mg is the drug of choice.

25.3 Cabergoline should be used with caution for women with hypertension or pre-eclampsia.

25.4 After a late miscarriage some women may choose to donate their milk to a breast milk bank. While discussing milk donation may be difficult, staff should sensitively offer women information about donating their milk.

26. WHO TO INFORM

26.1 All key staff responsible for care of the women during her pregnancy and afterwards should be informed of events.

26.2 All existing appointments should be cancelled.

26.3 With the woman's consent, a designated member of staff at the hospital should phone her GP and Community Midwife prior to the woman being discharged. This is important as it can take several days for letters or correspondence to be noted. The hospital should also immediately send a summary of the woman's pregnancy history and care to her GP when she is discharged. Dependant on the gestation at which the loss occurred, the summary should also be sent to the Community Midwife and the Health Visitor.

27. PSYCHOLOGICAL, SOCIAL AND SPIRITUAL ASPECTS OF CARE

27.1 Carers must be alert to the fact that mothers, partners and children can experience prolonged severe psychological reactions following mid-trimester loss. Carers should be aware of and responsive to possible variations in individual and cultural approaches to death.

27.2 Counselling should be offered to all women and their partners – this can be facilitated through the CWH counselling service up to four months following discharge. Contact details and information sheet is available in the bereavement folder and should be given to the parents on discharge. Parents can arrange their own appointment or a referral can be made on EPIC. A referral for counselling may be required in subsequent pregnancies.

27.3 Other family members, especially existing children and grandparents should also be considered for counselling – this would need to be facilitated through the family GP.

27.4 Parents should be advised about support groups. This is provided within the bereavement folders which should be offered to all parents.

27.5 Mortuary staff are available to coordinate and provide support to families in arranging funeral services.

27.6 The department of Pastoral, Religious and Spiritual Care offers support to all whether the parents have a religious faith or not and holds specific information on the needs of all the major world faith groups. A member of the team can come and meet parents as requested. Support may include a listening ear, a blessing, or naming ceremony for the baby or support for wider family members. Chaplains can also offer support or advice about funeral arrangements. Alternatively, women may wish to seek the support of their own minister or faith group leader.

Contacting the chaplaincy team

East: Ext 2024 or via switchboard for 24-hour on-call cover.
North: Ext 2362 until 10pm or via switchboard for transfer to East 24-hour on-call cover.

27.7 Staff should ensure the cultural and religious needs of parents are supported wherever possible. Staff should offer to contact the family's spiritual / religious support or hospital chaplaincy / spiritual service.

27.8 While grief does not necessarily result in mental health concerns, there may be cases where women or couples should be assessed for mental health concerns that may be compounding their grief. It can be helpful for staff to explain the symptoms of depression, anxiety and posttraumatic stress disorder. Parents should look out for these signs and seek further assistance.

28. SEEING, HOLDING, NAMING AND MEMENTOS

28.1 Parents should be offered information about what they might expect their baby to look like depending on the gestation and type of management used. Some parents may wish to see their baby and some may not. Carers should encourage parents to have contact with their baby whilst also demonstrating respect for their wishes as individuals. If parents wish to spend time with their baby this time should not be limited. Parents must have as much time as they need with their baby.

28.2 Parents may find making memories following the loss valuable and sharing these memories may be beneficial to the grieving process. Staff should offer parents opportunities to create memories where possible. Some women will appreciate the offer and others will not. Parents' choice is paramount.

28.3 Parents sometimes decide to name their baby or have a nick-name since early pregnancy. If so, staff should ask whether they would like them to use that name too. If they agree, remember to refer to the baby by that name during each contact.

28.4 Parents should be offered and supported, but not persuaded, to make mementos.

28.5 Memory boxes and a selection of mementos are available in the bereavement cupboard. Please contact the Bereavement Midwife if you are unsure. These should be offered to parents to create their own memories. No mementos or photographs can be stored in maternal hospital notes.

28.6 It should be explained to parents that clothes their baby is dressed in may become stained. Mortuary staff will launder clothes before redressing the baby but this may not remove all stains.

28.7 The loss of a baby or pregnancy before 24 weeks' gestation is not required or permitted by law to be officially certified or registered. Whilst some parents do not want to register a loss before 24 weeks, others may find this upsetting. Parents should be offered an unofficial certificate of birth, or certificate of pregnancy loss from the hospital but this should not be insisted upon if the parents decline.

The government has now launched a new baby loss certificate for parents experiencing a loss before 24 weeks of pregnancy. This can be requested by the Midwife on the parent's behalf prior to discharge or independently by the parents themselves.

To request a certificate Google: 'request a government baby loss certificate' and use the Gov.UK link to proceed. Request a baby loss certificate - GOV.UK (www.gov.uk)This is not compulsory - it remains the choice of all parents to manage the difficult time of a loss.

29. BEREAVEMENT PHOTOGRAPHY

29.1 Staff should offer to take photographs of the baby for the parents. The offer should be made only with great sensitivity. If parents decline photographs, photographs can still be taken and saved in the medical records in case the parents change their mind at a later date.

29.2 Multiple births

It is important that staff acknowledge the importance of the baby (or babies) who have died. Staff should offer parents, scan images of all the babies together, where possible. It may be helpful for any surviving siblings to see these images later on.

29.3 If photography is requested for clinical reasons, i.e. by Doctor / Geneticist these can be taken with the parent's written consent.

If the parents have signed to have a photograph taken by the hospital medical photographer and this has not been done before the baby is transferred to the mortuary, the consent form should accompany the baby and the mortuary staff will ensure that the photograph is taken before the baby goes for a post-mortem or is taken to a funeral director.

Contacting the Medical Photography Department

East: Service is only available during office hours and there may be a delay during weekends and bank holidays.

North: Currently not available.

30. DOCUMENTARY REQUIREMENTS FOLLOWING MID-TRIMESTER LOSS

30.1 Complete the 'Medical certificate from Doctor or registered Midwife' form and send for scanning into MyCare following discharge.

30.2 Some parents may wish to take their baby home. There are no legal reasons to prevent parents from taking their baby's body or remains home unless the death has been referred to the coroner. There may be clinical reasons (infection) and these need to be sensitively discussed with parents.

30.3 If parents wish to take their baby home, the 'Taking deceased baby home by parents' form should be completed on MyCare. This will need to be photocopied and provided for the parents and an additional copy sent to the mortuary with baby. Parents also need to be informed of the importance of keeping the body cool. They may be able to borrow a cold cot from the hospital, local group or hospice. Staff should know what is available before offering this option to parents.

30.4 Police must be contacted if there is suspicion of deliberate action to cause fetal death (Statute).

30.5 Complete a referral to the Bereavement Midwife on EPIC. Provide details of the loss and any other valuable information.

31. ROLE OF THE BEREAVEMENT MIDWIFE

31.1 All bereaved families should be given a single, named point of contact, who can provide information on the perinatal mortality review process, and who can signpost them to sources of support.

31.2 This role could be taken by a range of Practitioners, for example a Midwife or a member of a Bereavement Support Team. The qualities and competencies of the individual are more important than their professional background. Given for annual leave, Trusts should ensure that the Bereavement Midwife is supported by a team who can step in to cover absences. Families should expect to be able to contact the Specialist Bereavement Midwife or a team member during normal working hours.

Contact details:

RDUH Bereavement Midwife: Marie Charman – mariecharman@nhs.net Tel: 07741 386665.

East: Bereavement Team – rduh.rdebereavementservice@nhs.net Tel: 01392 402349.
North: Bereavement Team – rduh.nddhbereavementsupport@nhs.net Tel: 01271 322404.

31.3 Main Responsibilities

- Be a reliable and readily accessible point of contact for the family after the death.
- Help co-ordinate meetings between the family and professionals as required.
- Be able to provide information on the child death review process and the course of any investigations pertaining to the child, including liaising with the coroner's officer and any police family liaison officer.
- Represent the 'voice' of the parents at professional meetings, ensure that their questions are effectively addressed, and to provide feedback to the family afterwards; and
- Signpost to expert bereavement support if required.

31.5 Support for the Specialist Bereavement Midwife

1. **Time**. How much time will be needed for the role may vary greatly from case to case. It is important that all NHS organisations are flexible in enabling the Specialist Bereavement Midwife to support each individual family as required, over the weeks and months following the death of a child.
2. **Team support**. Families should expect to be able to contact the Specialist Bereavement Midwife or a team member during normal working hours. Given annual leave, Trusts should ensure that the Specialist Bereavement Midwife is part of a supportive team who can step in to cover absences.
3. **Individual support**. Working with bereavement can be stressful. The Specialist Bereavement Midwife and their line manager should agree a plan to ensure that they are appropriately supported in the role, including opportunities for debriefing and supervision.

The Specialist Bereavement Midwife will be appointed for each case in liaison with the Governance and Child Death Review Coordinator.

32. ROLE OF BEREAVEMENT TEAM

- To support women / parents in arranging funeral for their baby if required.
- To liaise between parents, mortuary and the undertaker.
- To inform women / parents on post-mortem pathway if undertaken.
- To support and advise the midwifery staff.

Contact the Mortuary Team
East: Ext. 6676
North: Ext. 2302

33. FUNERAL SERVICES

33.1 Parents should be encouraged to arrange a funeral for their baby regardless of how early in the pregnancy the loss occurred. The parents will be supported and given guidance on arranging a funeral for their baby. Parents will need to ensure that they inform the mortuary of the contact number of their chosen funeral directors.

33.2 The parents may not want or be able to arrange a funeral service. In this situation, the Trust will ensure the baby has a hospital cremation unless this is unacceptable on specific religious / cultural beliefs. An ‘Authorisation to Act on Parents’ Behalf’ form will need to be completed on EPIC and a printed copy sent to the mortuary with the baby.

Availability

East: Shared cremation only. Individual cremations must be arranged by the family.
North: Shared cremation or individual cremation are available subject to gestation.

33.3 Parents should be warned that if they choose to have their baby cremated, there may not be any ashes. Parents should be informed whether cremation options include individual or shared cremations.

33.4 Some women may decline this information or involvement in decision-making processes regarding the disposal of pregnancy remains and this should be respected. Parents should be informed of which option will be used if they do not decide, and told who they can contact if they change their minds or would like further information.

33.5 Parents should have the option of leaving toys, pictures and messages with their baby.

33.6 **Cultural considerations**

Staff should be aware that some cultures and religions do not traditionally hold funerals or other ceremonies for babies born before 24 weeks' gestation. It should not be assumed however that this is the case, and all parents should be offered information on the disposal methods and options for hospital or private funerals available locally. The chaplaincy team are available to discuss any issues and put them in touch with a representative of their faith if they have one.

34. BOOK OF REMEMBRANCE

34.1 **East**:

A Book of Remembrance is available for parents to enter the details of their baby. Parents should be given a Book of remembrance form to complete and return. The details will be written into the remembrance book and the parents should be informed that they can come and look at the Book of Remembrance at any time. The book of remembrance is kept in the Willow Room.

34.2 **North:**

A Book of Remembrance is available for parents to enter the details of their baby. Please contact Jonathon Daniel. The book is kept with the Chaplain.

35. FERTILITY ADVICE

35.1 Information about fertility and contraception should be offered to all mothers before going home.

35.2 Mothers should be advised that they are vulnerable to psychological disorders when conception occurs soon after the loss.

35.3 With suppression of lactation, ovulation returns more quickly and can be as early as day 18. Women should be advised that they could conceive before their first menstrual cycle.

36. FOLLOW UP

36.1 The wishes of the woman and her partner should always be considered when arranging a follow-up.

36.2 After the post mortem, the parents should have a follow-up appointment with her named Consultant to discuss the results when they are all available. This will usually take a minimum of 6 weeks but may take up to several months. If there is a delay to the post mortem results, it is important that this information is shared with the healthcare team supporting the parents and that this information is conveyed to the parents as soon as possible.

36.3 At the follow-up meeting, parents should be advised of the following:

- Where possible the cause of mid-trimester loss, chance of recurrence and any specific means of preventing a further loss.
- Offered general pre-pregnancy advice such as smoking cessation and weight loss.
- Offer to discuss potential benefits of delaying conception until severe psychological issues have resolved.
- Advised that the chance of adverse effects with a pregnancy interval less than 6 months remains low and is unlikely to be significantly increased compared with conceiving later.

36.4 The meeting should be documented for the parents in a letter that includes an agreed outlined plan for future pregnancy.

36.5 Additional follow-up appointments should be made to talk through what happened, to share information, and to discuss next steps. Some women will appreciate this opportunity, some may find it too stressful.

37. FUTURE PREGNANCY

- The history of mid-trimester loss must be clearly documented within the medical records and should be thoroughly read before seeing the woman.
- Parents should feel supported in any pregnancy following a pregnancy loss or death of a baby. All staff must be aware of and acknowledge the potential difficulties and challenges these parents might be facing.
- Parents may also worry about the effects of the fear and anxiety they are feeling on the wellbeing of this baby. It is important that staff acknowledge the validity of parents' concerns and take them seriously.
- Women should be referred to a Consultant for antenatal care.
- Women should be recommended to have screening for gestational diabetes.
- For women whose baby showed evidence of being small for gestational age, serial assessment of growth by USS should be recommended.

38. FUTURE BIRTH

- Women with a previous unexplained mid-trimester loss should be recommended to birth at a Specialist Maternity Unit.
- Women with a previous mid-trimester loss related to a known non-recurrent cause should have their place of birth individually assessed.
- Maternal request for a scheduled IOL should consider the gestational age of the previous mid- trimester loss, previous intrapartum history and the safety of IOL.

39. MATERNAL CARE FOLLOWING THE NEXT BIRTH

- Carers should be vigilant in observing for postpartum depression in women with a previous mid- trimester loss.
- Carers should be aware that maternal bonding can be adversely affected in future pregnancies.

40. ASSOCIATED CLINICAL GUIDELINES OR POLICIES

- Please see Hub for other pregnancy loss / neonatal death guidelines specific to gestation.

41. MONITORING COMPLIANCE WITH THIS GUIDELINE

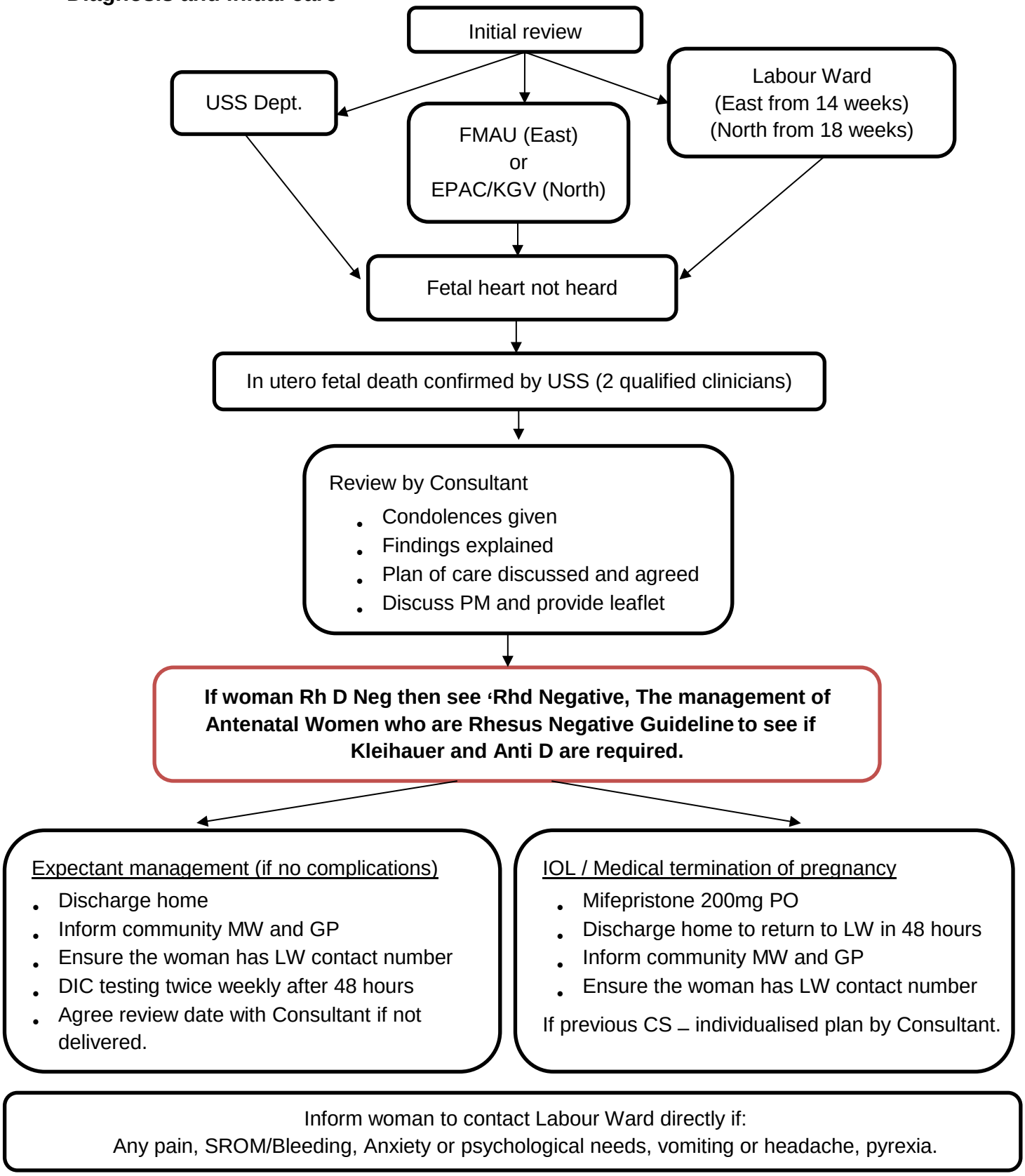
1. This guidance will be monitored and reviewed on a triennial basis by the Women's and Children's Health Care Group / Maternity.
2. The MBRRACE guidance and pregnancy losses relevant to the PMRT criteria will be reviewed at the Governance meetings.

42. PUBLICATION DETAILS

Author of Clinical Guideline	Marie Charman – Specialist Bereavement Midwife.
Division/ Department responsible for Clinical Guideline	Women's and Children's Health Care Group / Maternity
Contact details	mariecharman@nhs.net
Version number	V1.1
Replaces version number	Previous guidance from both the Eastern and Northern sites.
Date written	2024 merged both sites guidance
Consultation undertaken with:	Northern and Eastern sites: Obstetric team Midwifery team Mortuary Bereavement teams Neonatal teams Chaplaincy
Approving body and date approved	Maternity Guideline Group 09/04/2025 *(minor changes throughout)*
Review date	09/01/2030
Expiry date	09/04/2030
Date document becomes live	15/04/2025

APPENDIX 1 - Care pathway for women experiencing mid trimester loss

Diagnosis and initial care



APPENDIX 2 - Labour Care Pathway for Women Experiencing Mid-Trimester Loss

Care for Labour and Birth

Admit to bereavement room ensuring a birthing bed is used.

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Commence paperwork and EPIC pregnancy loss actions

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Ensure that a bedpan is placed in the toilet at all times as delivery could occur at any time from admission

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Drugs for labour <24 weeks in unscarred uterus.

- Mifepristone 200mg – **Followed 48 hours later by**
- Misoprostol 400 mcg PV/SL/PO 4 hrly until delivery

↓

Drugs for labour for women with 1 x previous LSCS

- Mifepristone 600mg PO for 2/7 - **followed 48 hours later by**
- Misoprostol 400mcg PV 4 hourly with senior review after 4 doses

↓

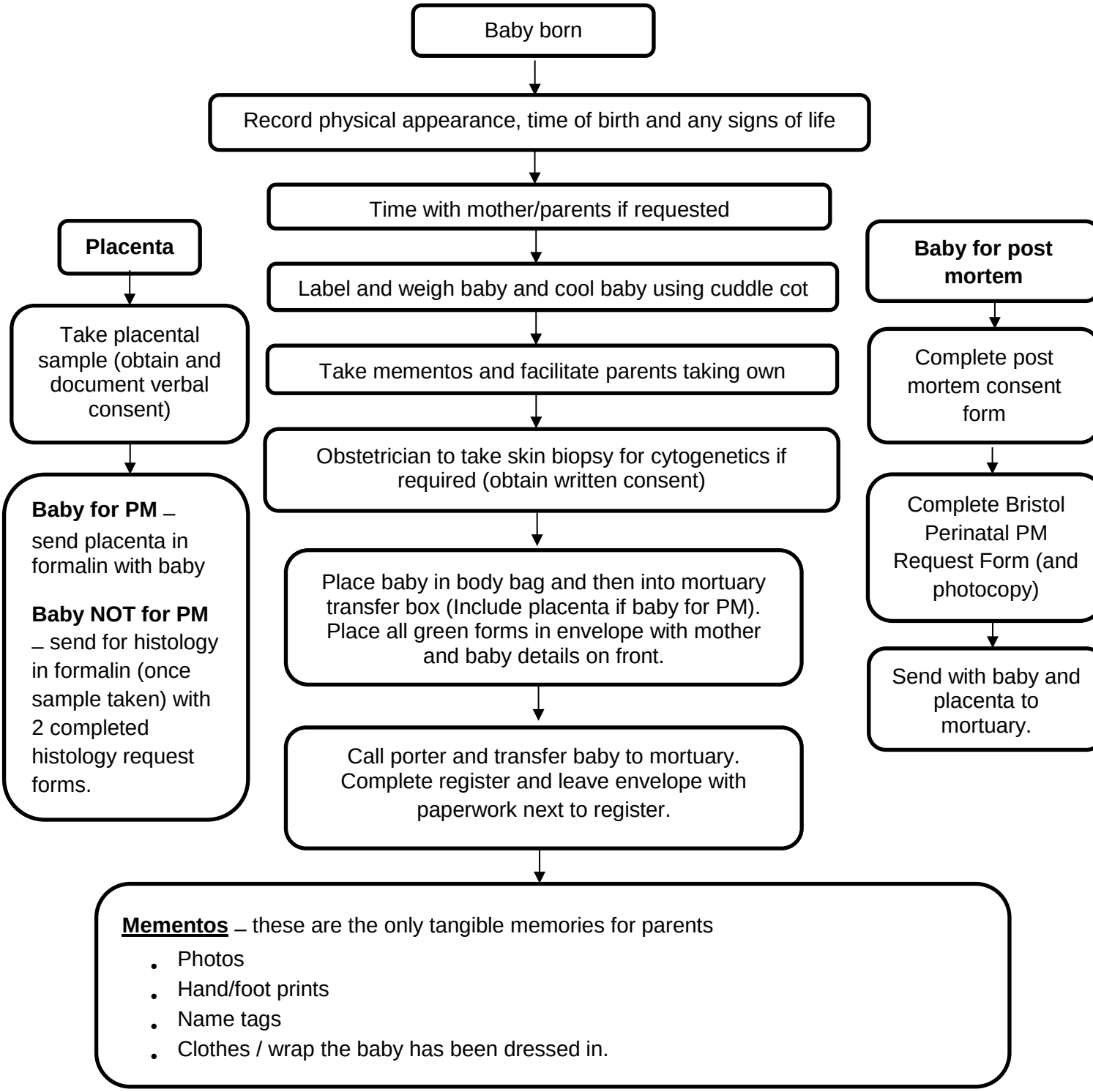
- **Normal labour care** – Be aware that labour may progress rapidly and birth can occur suddenly once contractions commence.

Women undergoing VBAC should be closely monitored for features of scar rupture

Adequate analgesia should include:

  - Simple analgesia
  - Entonox
  - Opioids – Diamorphine
- Birth as normal. Vaginal breech birth experienced Midwife / SpR present.
- Strongly recommend Active 3rd stage for all women over 14 weeks gestation.

APPENDIX 3 - Care Once Baby Born



APPENDIX 4

FUNERAL ARRANGEMENTS

EAST
Parents are required to make their own funeral arrangements for either a burial or cremation. The hospital will only arrange in exceptional circumstances and by agreement of a Midwifery Manager.

NORTH
Parents are encouraged to make their own funeral arrangements for either burial or cremation. The hospital can arrange burial or cremation if requested.

Complete the stillbirth certificate.
Send to the mortuary with baby. The team will contact the registrar of Births/Deaths/Marriages.

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Mother or parents register the stillbirth. The BD8 green form will be given to their chosen undertaker by the bereavement team.

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Complete the 'Authorisation for release of deceased baby' form and send with the baby and other paperwork to the mortuary.

Cremation - Parents choosing a cremation should be informed that there may not be any remains (ashes) due to the size of their baby.

Certificate of cremation – A Crem 9 form must be completed and sent to the mortuary with the baby. **Please note – this is not the stillbirth certificate required for registering the stillbirth.**

If parents are undecided on their final decision:

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- Give them as much information as possible.
- Tell them they can take as much time as they need to decide.
- Tell them to contact the mortuary when they have decided.
- Tell them that the mortuary staff will contact them in a few days regarding the funeral arrangements.

APPENDIX 5 - PROTOCOL FOR TAKING CYTOGENETIC SPECIMENS FROM A BABY

The baby should remain on Labour Ward until the mother / parents are ready to say goodbye.

All specimens for cytogenetics should be taken, by the responsible clinician, and sent to the Genetics Lab at Southmead Hospital. There is a folder on Labour Ward with picture guides for Lab tests and how to post the placental biopsies.

One of the following should be sent:

Skin Biopsy

A full explanation of the procedure and reason for sample must be given to the mother or parents and verbal consent must be gained and written on the cytogenetics form and clearly documented on EPIC.

It should be explained that it is more likely to get a result from a skin sample than from a placental sample.

This sample is taken (by the Registrar / SHO), from the fleshy part of the thigh or upper arm, using an aseptic technique.

- Rinse the skin with sterile normal saline.
- Cut an ellipse of skin 1 cm long, together with underlying muscle.
- Place biopsy in sterile universal container once thawed (kept in Del suite freezer).
- Send to Blood Bank immediately together with completed cytogenetics request form.
- Out of hours. Refrigerate overnight at 4 0 C.

OR

Placental tissue

A full explanation of the procedure and reason for sample must be given to the mother or parents and verbal consent must be gained and written on the cytogenetics form and clearly documented in EPIC.

Parents should be informed that a result is less likely from placental tissue and there is a risk of placental mosaicism.

- Using a sterile technique, take a 3 cm sample of placenta from the fetal side, close to the cord insertion, to include amnion, placenta and cord.
- Place biopsy in sterile universal container once thawed (kept in Del Suite freezer).
- Send to Blood Bank immediately together with completed cytogenetics request form.
- Out of hours. Refrigerate overnight at 4 0 C.

DO NOT send these samples to the mortuary with the baby!

APPENDIX 6

INDUCTION REGIME FOR INTRAUTERNINE DEATH/MEDICAL TERMINATION OF PREGNANCY - May 2024

	Unscarred Uterus			1 x Previous Section		
Gestation	**14+0 - 23+6**	**24+0 – 27+6**	**> 28**	**14+0 -23+6**	**24+0- 27+6**	**>28**
Mifepristone 48 hours prior to admission	200mg orally	200mg orally	200mg orally	600mg orally once daily for 2 days.	600mg orally once daily for 2 days.	600mg orally once daily for 2 days.
Misoprostol Dose	400 mcg	200 mcg	100 mcg	400 mcg	200 mcg	50 mcg Angustatablets (2x 25mcg tablets)
Route	PV/SL/PO	PV/SL/PO	PV/SL/PO	PV/SL/PO	PV/SL/PO	PO
Frequency	4 hrly until delivery	4 hrly until delivery	4 hrly until delivery	4 hrly for 4 doses then review	4 hrly for 4 doses then review	4 hrly for 4 doses then review
Please also prescribe:	Paracetamol PRN 1g orally, 4-6 hourly, max 4g in 24 hrs Dihydrocodeine PRN 30mg orally, 6 hourly Diamorphine 5mg , 4 hourly Prochlorperidine 12.5 mg IM, 8 hourly These drugs have to be prescribed as they are only part of a Midwives' exemption if a woman is in normal established labour.					

Note:

- The uterus is more sensitive to Misoprostol as pregnancy advances
- Risk factors for uterine rupture include a pre-existing uterine scar, increased gestation age and multi-parity Pyrexia and shivering are common side effects of Misoprostol. The pyrexia lasts for a few hours and will not respond to paracetamol

2 or more previous LSCS – individual plan of care made by the Consultant.